

A mathematical modelling of rheumatoid arthritis

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Abstract

We model the interaction between three populations of cells in the context of arthritis, namely the neutrophils, the tissue resident macrophages (SubLining Tissue Resident Macrophages) and the Monocyte Macrophages. The SL-TRM macrophage cells reside in the outer border of the joint. The Momac (monocytes-macrophages) cells are coming from the reservoir of monocytes in the marrow.

We propose a minimal ordinary differential equation model of acute inflammatory response in a joint, describing interactions between neutrophils, lining tissue-resident macrophages (SL-TRM), and recruited monocyte-derived macrophages (Momac). The model couples an inflammatory signal S to three cell populations (N , T , M) and incorporates recruitment, conversion, nonlinear self-limitation and mutual regulation. By nondimensionalization we show that the equilibrium structure depends on three adimensional parameters A , B , C and derive a cubic equation characterizing nontrivial steady states. We perform an analytical study of equilibria and their stability, identify a bifurcation at $A = 1$ separating disease-free and inflamed regimes, and discuss conditions for multiple steady states and biological feasibility. Numerical simulations illustrate typical transient dynamics and phase-space geometry. The model provides a simple framework to explore mechanisms of chronic inflammation, guide parameter estimation from data, and extend to control or treatment scenarios.

1 Model

Basically the crisis scenario is the following: The arthritis crisis is triggered by an external signal (injection of immune serum containing antigens) that activates the immune system and provokes an inflammatory response. The antigens are recognized by the macrophages L-TRM (lining tissue resident macrophages) cells that secrete inflammatory chemokines and cytokines. These cytokines activate the fibroblasts which in turn recruit the neutrophils cells from the blood stream into the joint. The cytokines also recruit the Momac cells from the blood stream into the joint. There is a positive feedback since the inflammation is amplified by the neutrophils that are recruited in the joint by the chemokines

secreted by the Momac cells. The macrophages SL-TRM cells have a protective role since they inhibit the inflammation but they are depleted during the crisis because they are inhibited by the proliferation of Momac cells.

1.1 Mathematical Model

Let us denote by $T(t)$ the population of SL-TRM cells and by $M(t)$ the population of Momac cells at time t . The cells T (SL-TRM) and M (Momac) interact with each other in the following way. The Momac cells M are recruited from the blood stream at a constant rate a . They die at a rate δ_M and they transform into SL-TRM cells at a rate b . The SL-TRM cells T die at a rate δ_T . The interaction between the two populations is described by the following system of differential equations

The inflammatory state $S(t)$ is enhanced by neutrophils N and inhibited by SL-TRM macrophages T .

Inflammation further increases neutrophils (factor α), increases Momac (factor β), and decreases SL-TRM (factor γ).

Notice that we do not keep track of the population of fibroblasts, but rather the population of neutrophils, which are recruited by inflammatory chemokines.

The variable $N(t)$ designates the population of neutrophils at time t . Finally $S(t)$ is the inflammatory signal at time t . The model is given by the following system of ordinary differential equations:

$$\begin{cases} \frac{dS}{dt} = (a N - b T)S \\ \frac{dN}{dt} = \alpha S N - \delta_N N(N - 1) \\ \frac{dM}{dt} = \beta S - \delta_M M(M - 1) \\ \frac{dT}{dt} = -\gamma S T + \delta_M M(1 - T) \end{cases} \quad (1)$$

where $a, b, \alpha, \beta, \gamma, \delta_N, \delta_M$ are positive parameters. To make things easier to understand, we summarize the model in the following diagram (see Figure 1.1).

Comments and questions on the model:

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2 Analysis of the model

2.1 Equilibria points

Equilibria points of the system are found by setting the right-hand side of the system to zero, i.e. by solving the following system of equations: The equilibrium points (S^*, N^*, M^*, T^*) satisfy the following system:

$$(aN^* - bT^*)S^* = 0 \quad (2)$$

$$\alpha S^* N^* - \delta_N N^*(N^* - 1) = 0 \quad (3)$$

$$\beta S^* - \delta_M M^*(M^* - 1) = 0 \quad (4)$$

$$-\gamma S^* T^* + \delta_M M^*(1 - T^*) = 0 \quad (5)$$

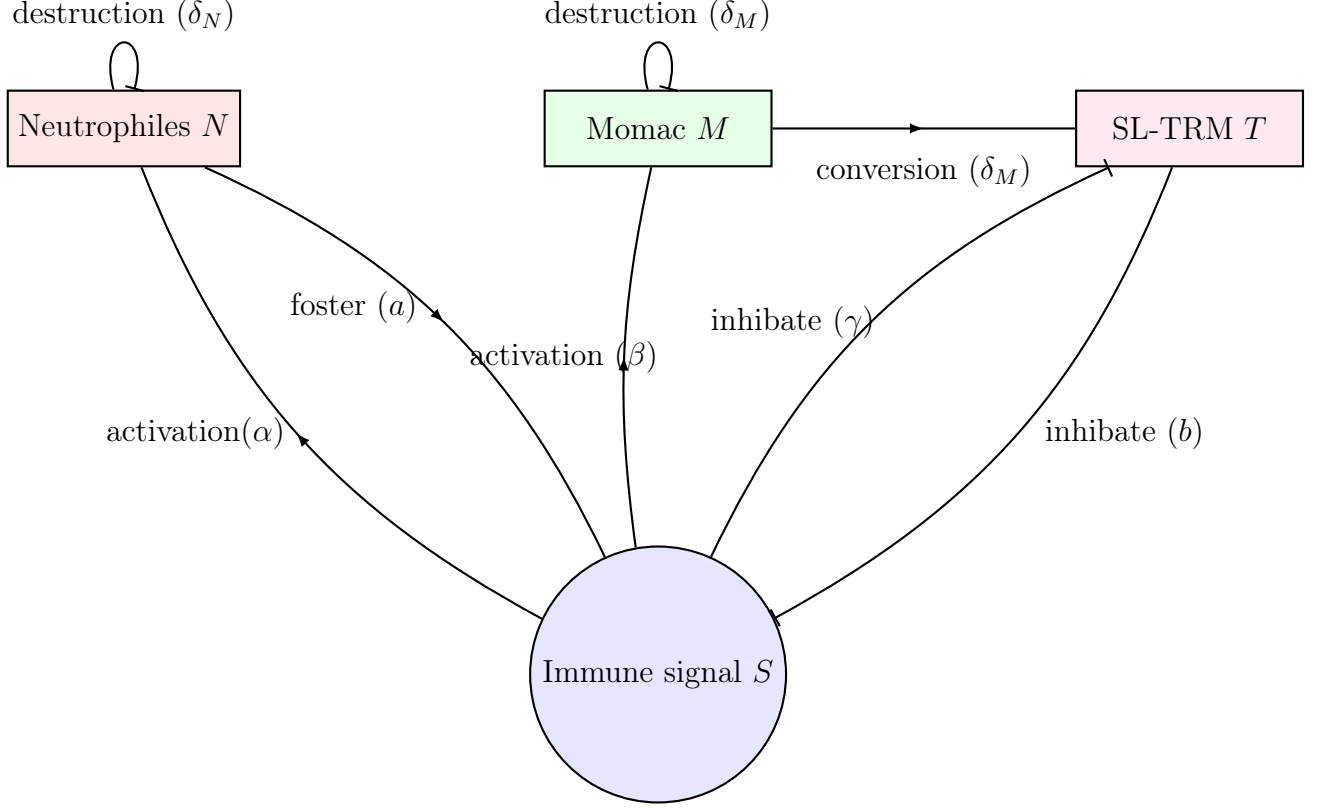


Figure 1: Schematic representation of the model

2.1.1 Inflammation free equilibria ($S^* = 0$).

A first natural equilibrium point is given by $S^* = 0, N^* = 1, M^* = 1, T^* = 1$. This point corresponds to the disease-free state, where there is no inflammation ($S^* = 0$) and the populations of neutrophils, Momac, and SL-TRM are at their baseline levels ($N^* = 1, M^* = 1, T^* = 1$).

There are other equilibria points with $S^* = 0$ namely $S^* = 0, N^* = 0, M^* = 0, T^* = R$, with arbitrary R ; $S^* = 0, N^* = 0, M^* = 1, T^* = 1$; $S^* = 0, N^* = 1, M^* = 0, T^* = R$, with arbitrary R .

Remark 2.1 *These equilibria points with $S^* = 0$ and $N^* = 0$ or $M^* = 0$ are not biologically relevant since they correspond to the absence of neutrophils or Momac cells, which is not realistic in a living organism. Anyway, we will prove later that these equilibria points are unstable, so they will not be observed in practice.*

2.1.2 Equilibria points with $S^* \neq 0$.

We express N^* in terms of T^* from equation (2):

$$N^* = \frac{b}{a} T^* \quad (6)$$

Substituting this expression for N^* into equation (3), we obtain:

$$S^* = \frac{\delta_N}{\alpha} \left(\frac{b}{a} T^* - 1 \right) \quad (7)$$

From equation (4) and equation (5), we derive $\delta_M M^* = \frac{\beta S^*}{M^* - 1}$ and $\delta_M M^* = \frac{\gamma S^* T^*}{1 - T^*}$. Equating these two expressions for $\delta_M M^*$, we obtain:

$$\frac{\beta S^*}{M^* - 1} = \frac{\gamma S^* T^*}{1 - T^*}$$

Assuming $S^* \neq 0$, we can simplify this equation to:

$$\frac{\beta}{M^* - 1} = \frac{\gamma T^*}{1 - T^*}$$

From this, we can express M^* in terms of T^* :

$$M^* = 1 + \frac{\beta(1 - T^*)}{\gamma T^*} = 1 - \frac{\beta}{\gamma} + \frac{\beta}{\gamma T^*} \quad (8)$$

Substituting the expression for M^* and S^* into the last equation (5), we obtain

$$-\gamma \frac{\delta_N}{\alpha} \left(\frac{b}{a} T^* - 1 \right) T^* + \delta_M \left(1 - \frac{\beta}{\gamma} + \frac{\beta}{\gamma T^*} \right) (1 - T^*) = 0 \quad (9)$$

Since $T^* \neq 0$, we can multiply this equation by T^* and rearrange the terms to obtain a cubic equation for T^* :

$$\frac{\gamma}{\alpha} \frac{\delta_N}{\delta_M} \frac{b}{a} (T^*)^3 + \left(1 - \frac{\gamma}{\alpha} \frac{\delta_N}{\delta_M} - \frac{\beta}{\gamma} \right) (T^*)^2 + \left(2 \frac{\beta}{\gamma} - 1 \right) T^* - \frac{\beta}{\gamma} = 0 \quad (10)$$

Rewriting this equation in the standard unitary form $(T^*)^3 + p(T^*)^2 + qT^* + r = 0$, we have:

$$(T^*)^3 + \frac{\alpha}{\gamma} \frac{\delta_M}{\delta_N} \frac{a}{b} \left(1 - \frac{\beta}{\gamma} - \frac{\gamma}{\alpha} \frac{\delta_N}{\delta_M} \right) (T^*)^2 + \frac{\alpha}{\gamma} \frac{\delta_M}{\delta_N} \frac{a}{b} \left(2 \frac{\beta}{\gamma} - 1 \right) T^* - \frac{\alpha}{\gamma} \frac{\delta_M}{\delta_N} \frac{a}{b} \frac{\beta}{\gamma} = 0 \quad (11)$$

Notice that we need only 3 *non dimensional* parameters a/b , α/β , $\frac{\gamma \delta_N}{\alpha \delta_M}$ instead of the 7 original parameters. In the following, we denote the 3 *non dimensional* parameters as $A = a/b$, $B = \beta/\gamma$, and $C = \frac{\gamma \delta_N}{\alpha \delta_M}$.

Remark 2.2 *The parameters A , B , and C have the following biological interpretations:*

- $A = a/b$. *This parameter represents the ratio of the inflammatory effect of the neutrophils w.r.t. the anti-inflammatory effect of the macrophages.*
- $B = \beta/\gamma$. *This parameter represents the ratio of the increasing rate of the Momac to the decreasing rate of the macrophage.*
- $C = \frac{\gamma \delta_N}{\alpha \delta_M} = \left[\frac{\gamma}{\delta_M} : \frac{\alpha}{\delta_N} \right]$. *This parameter represents the ratio of the macrophages vs the neutrophiles effects in the disease progression.*

Using these parameters, the cubic equation in T^* becomes:

$$(T^*)^3 + \frac{A}{C} (1 - B - C) (T^*)^2 + \frac{A}{C} (2B - 1) T^* - \frac{AB}{C} = 0 \quad (12)$$

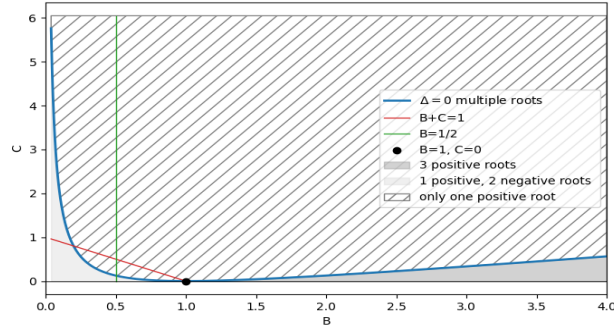


Figure 2: $A = 1$, number of roots.

Remark 2.3 *There is an obvious solution in the particular case $A = a/b = 1$, namely $T^* = 1$. This solution corresponds to the disease-free equilibrium point $S^* = 0, N^* = 1, M^* = 1, T^* = 1$. But this solution is not relevant for the case $S^* \neq 0$ that we are considering here. Note further that if $T^* = 1$ then from Eq. (12) we get*

$$1 + \frac{A}{C}(1 - B - C) + \frac{A}{C}(2B - 1) - \frac{AB}{C} = 0$$

so that $A = 1$ hence $T = 1$ is a root of the cubic equation (12) iff $A = 1$. Therefore we can factor the cubic polynomial as follows:

$$(T^* - 1) \left((T^*)^2 - \frac{1}{C}(B - 1)T^* + \frac{B}{C} \right) = 0 \quad (13)$$

- If $B \leq 1$, then the coefficients of the quadratic factor are positive, hence there are no other positive roots.
- If $B > 1$, the quadratic factor has either two positive roots (if the discriminant is positive) or no real roots (if the discriminant is negative). The quadratic factor has discriminant $\Delta = \frac{(B-1)^2 - 4BC}{C^2}$. Thus, we have:

– If $\Delta > 0$, there are two distinct real roots given by:

$$T^* = \frac{\frac{(B-1)}{C} \pm \sqrt{\Delta}}{2}$$

– If $\Delta = 0$, there is one (double) positive root given by:

$$T^* = \frac{B - 1}{2C}$$

– If $\Delta < 0$, there are no real roots.

The sign of the discriminant of the quadratic factor depends on the parameters B and C : $\Delta > 0$ iff $\frac{(B-1)^2}{4B} > C$. Let us plot the curve $\frac{(B-1)^2}{4B}$ in the (B, C) plane (see Figure 2). Looking at the figure 2, we see that the typical situation is when $\Delta < 0$, hence there are no other positive roots apart from $T^* = 1$. But for a careful choice of parameters B and C , we can indeed have three equilibria, see fig. 3.

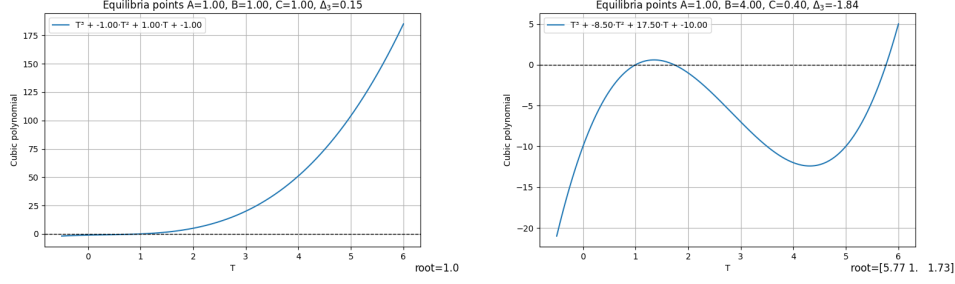


Figure 3: $A = 1$, Left 1 positive root, Right 3 positive roots

Returning to the general case $A \neq 1$, we analyze the number of positive roots of the cubic equation (12). From basic algebra, we can show that there are at most 3 such intersection points, hence at most 3 equilibria points with $S^* \neq 0$. We can be more precise because the biological feasibility imposes the roots to be real nonnegative numbers. Since $\frac{AB}{C} > 0$, there is at least one positive root. But we can be more precise using *Descartes' rule of signs*. It states that the number of positive real roots of a polynomial (counting multiplicities) is equal to the number of sign changes between consecutive nonzero coefficients, or less than that by an even number. In our case, the coefficients of the cubic polynomial in (12) are:

$$1, \quad \frac{A}{C}(1 - B - C), \quad \frac{A}{C}(2B - 1), \quad -\frac{AB}{C}$$

So the number of sign changes depends on the signs of the second and third coefficients. We have the following cases:

- If $B + C > 1$ and $B > 1/2$, then there are 3 sign changes and the cubic has one or three positive roots, which can collapse according to their multiplicities.
- If $B + C < 1$ and $B > 1/2$, then there is one sign change and the cubic has only one positive roots.
- If $B + C < 1$ and $B < 1/2$, then there is one sign changes and the cubic has only one positive root.
- If $B + C > 1$ and $B < 1/2$, then there is one sign change and the cubic has only one positive root.

We can plot some typical cases in the following figure (see Figure 4).

We can use Cardano's method to find the roots of the cubic equation (12). We rewrite it in the standard form:

$$T^3 + pT^2 + qT + r = 0$$

where

$$p = \frac{A}{C}(1 - B - C), \quad q = \frac{A}{C}(2B - 1), \quad r = -\frac{AB}{C}$$

We perform the change of variable $T = x - p/3$ to eliminate the quadratic term and obtain the depressed cubic equation:

$$x^3 + \mathcal{A}x + \mathcal{B} = 0$$

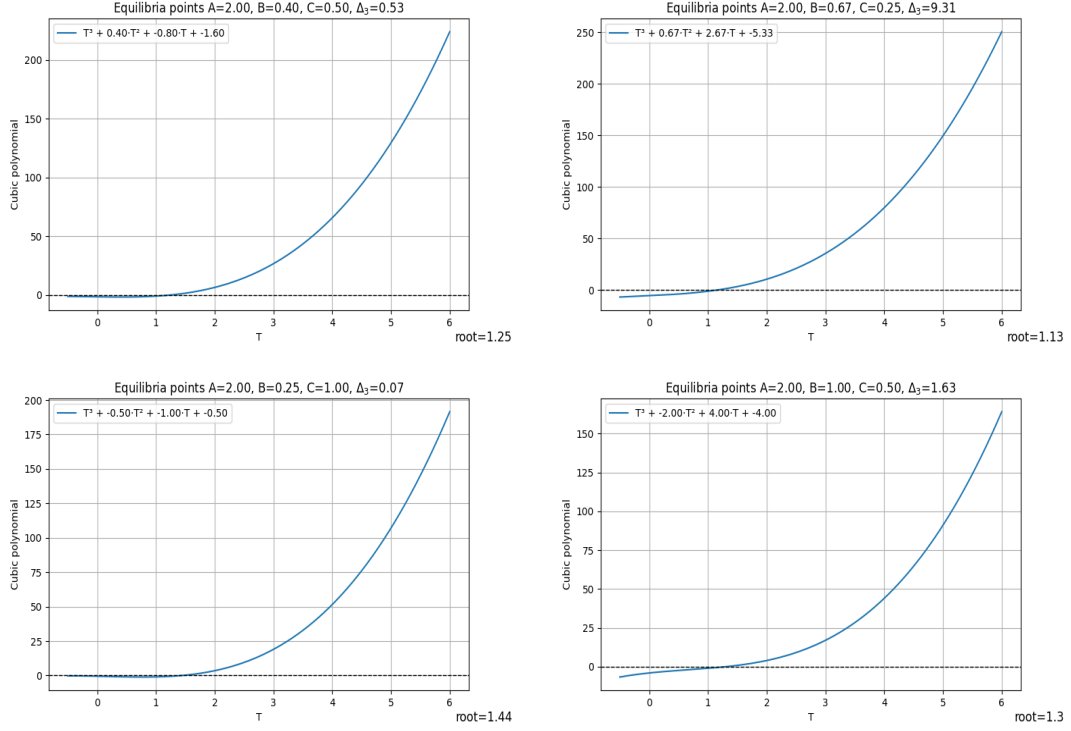


Figure 4: Cubic equation in T^* , left $B < 1/2$, right $B > 1/2$.

where

$$\mathcal{A} = q - \frac{p^2}{3}, \quad \mathcal{B} = \frac{2p^3}{27} - \frac{pq}{3} + r$$

The discriminant of this equation is given by:

$$\Delta_3 = \left(\frac{\mathcal{B}}{2}\right)^2 + \left(\frac{\mathcal{A}}{3}\right)^3$$

The nature of the roots depends on the sign of the discriminant Δ :

- If $\Delta_3 > 0$, there is one real root and two complex conjugate roots.
- If $\Delta_3 = 0$, all roots are real and at least two are equal.
- If $\Delta_3 < 0$, all roots are real and unequal.

Hence we can characterize the case when we have three distinct real roots by the condition $\Delta_3 < 0$, which is equivalent to:

$$\left(\frac{\mathcal{A}}{3}\right)^3 < -\left(\frac{\mathcal{B}}{2}\right)^2$$

This condition can be rewritten in terms of the original parameters using computer algebra as follows:

$$\left(\frac{\mathcal{A}}{3}\right)^3 < -\left(\frac{\mathcal{B}}{2}\right)^2$$

where

$$\mathcal{A} = \frac{A}{3C^2} \left(-A(B+C-1)^2 + 3C(2B-1) \right)$$

$$\mathcal{B} = \frac{A}{27C^3} \left(-2A^2(B+C-1)^3 + 9AC(2B-1)(B+C-1) - 27BC^2 \right)$$

$$\Delta_3 = \frac{A^2}{2916C^6} \left(-4A(A(B+C-1)^2 - 3C(2B-1))^3 + (2A^2(B+C-1)^3 - 9AC(2B-1)(B+C-1) + 27BC^2)^2 \right)$$

The real roots can be found using Cardano's formulas. If $\Delta_3 > 0$, the real root is given by:

$$x^* = \sqrt[3]{-\frac{\mathcal{B}}{2} + \sqrt{\Delta_3}} + \sqrt[3]{-\frac{\mathcal{B}}{2} - \sqrt{\Delta_3}}$$

If $\Delta_3 < 0$, the three real roots are given by:

$$x_k^* = 2\sqrt{-\frac{\mathcal{A}}{3}} \cos \left(\frac{1}{3} \arccos \left(\frac{3\mathcal{B}}{2\mathcal{A}} \sqrt{-\frac{3}{\mathcal{A}}} \right) + \frac{2k\pi}{3} \right), \quad k = 0, 1, 2$$

If $\Delta_3 = 0$, the real roots are given by:

$$x_1^* = 2\sqrt[3]{-\frac{\mathcal{B}}{2}}, \quad x_2^* = x_3^* = \sqrt[3]{\frac{\mathcal{B}}{2}}$$

Here we plot a case with three distinct positive roots (see Figure 5). Namely we take $A = 1.1$, $B = 5$ and $C = \frac{2}{3}$.

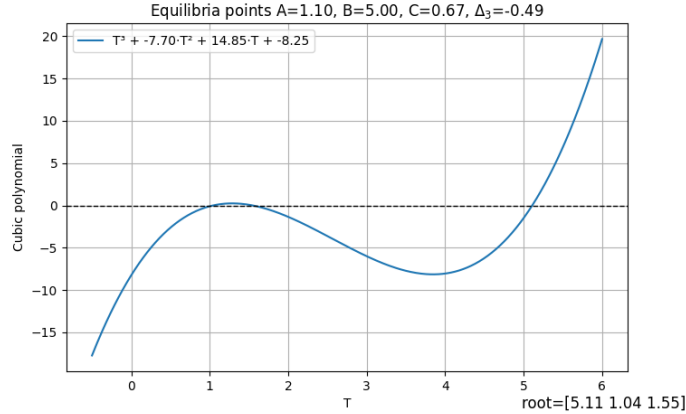


Figure 5: Cubic equation with three distinct real roots.

Finally, we can recover the roots T^* of the original cubic equation using the relation $T^* = x^* - p/3$.

We have to make sure that the nonnegative roots T^* found are biologically feasible, i.e. that they lead to nonnegative values of S^* , N^* , and M^* . For instance in the case $A = 1.1$, $B = 5$, $C = 2/3$ plotted in Figure 5, we have three distinct positive roots of the cubic equation (12), but none satisfying the biological feasibility conditions.

From equations (6), (7), and (8), we have:

$$N^* = \frac{b}{a}T^*, \quad S^* = \frac{\delta_N}{\alpha} \left(\frac{b}{a}T^* - 1 \right), \quad M^* = 1 - \frac{\beta}{\gamma} + \frac{\beta}{\gamma T^*}$$

Thus, the conditions for biological feasibility are:

- $S^* \geq 0$ iff $T^* \geq \frac{a}{b} = A$
- $N^* \geq 0$ iff $T^* \geq 0$ (always true since we consider only positive roots)
- $M^* \geq 0$ iff $T^* \leq \frac{\beta}{\beta-\gamma} = \frac{B}{B-1} = 1 + \frac{1}{B-1}$ if $B = \frac{\beta}{\gamma} > 1$; always true if $B \leq 1$.

So the biological feasibility conditions can be summarized as follows:

$$T^* \geq A \quad (14)$$

$$T^* \leq 1 + \frac{1}{B-1} \quad \text{if } B > 1 \quad (15)$$

Let us consider again the particular case $A = a/b = 1$, if $B \leq 1$ there is only one equilibrium, the disease-free $(S^*, N^*, M^*, T^*) = (0, 1, 1, 1)$ otherwise if $B > 1$ the feasibility conditions (14)-(15) reduce to:

$$1 \leq T^* \leq 1 + \frac{1}{B-1}. \quad (16)$$

We can check easily that only the disease-free root $T^* = 1$ satisfies this condition, while the other two roots are outside this interval (see Figure 3). Indeed if $\Delta \geq 0$, we have $C \leq \frac{(B-1)^2}{4B}$, hence $\frac{B-1}{2C} \geq \frac{B-1}{2 \cdot \frac{(B-1)^2}{4B}} = \frac{2B}{B-1} \geq 2$ so the root $T^* = \frac{B-1+\sqrt{\Delta}}{2C} \geq 2$ is not feasible.

Moreover, the other root $T^* = \frac{B-1-\sqrt{\Delta}}{2C} \leq \frac{B-1}{2C}$. In order to be feasible, this root should satisfy $\frac{B-1-\sqrt{\Delta}}{2C} \leq \frac{B}{B-1}$, which implies $C \geq \frac{(B-1)^2}{2B}$, contradicting the assumption $\Delta \geq 0$.

In the general case $A \neq 1$, the feasibility conditions (14)-(15) have to be checked for each positive root found. For $A > 1$, it will be shown that none of the positive roots satisfy the feasibility conditions (see Figure 5) whereas for $A < 1$, there is always exactly one feasible root (in addition to the disease-free equilibrium) (see Figure 6). Proving this, there are four cases to consider depending on the signs of $A - 1$ and $B - 1$.

1. case $A > 1$ and $B \geq 1$.

Let us define $f(T^*)$ as the cubic polynomial in the left-hand side of equation (12).

If $B > 1$ we have

$$\begin{aligned} f(T^*) &= T^{*3} - \frac{A}{C}(B-1)T^{*2} - AT^{*2} + \frac{A}{C}(B-1)T^* + \frac{A}{C}BT^* - \frac{AB}{C} \\ &= \frac{A}{C}(B-1)\left(T^* - \frac{B}{B-1}\right)(1 - T^*) + T^{*2}(T^* - A) \end{aligned}$$

So $f(T^*) > 0$, for $T^* \in [A, \frac{B}{B-1}]$ if $B > 1$.

If $B = 1$ then $f(T^*) = T^{*2}(T^* - A) + \frac{A}{C}(T^* - 1) > 0$ for $T^* > A$.

Hence, thanks to (14) and (15), there is no feasible root in this case.

2. case $A > 1$ and $B < 1$. As

$$f(T^*) = \frac{A}{C}(B-1)\left(T^* - \frac{B}{B-1}\right)(1 - T^*) + T^{*2}(T^* - A) > 0$$

for $T^* \geq A$ and $B < 1$, there is no feasible root in this case either.

3. case $A < 1$ and $B \geq 1$.

We compute $f(1) = 1 - A > 0$ and $f(A) = (A - 1)\frac{A}{C}(A(1 - B) + B) < 0$, so there is at least one feasible root in this case.

If $B > 1$, computing the derivative $f'(T^*)$ we have:

$$f'(T^*) = \frac{A}{C}(B - 1)\left[\left(\frac{B}{B - 1} - T^*\right) + (1 - T^*)\right] + 2T^*(T^* - A) + T^{*2} > 0$$

for $T^* \in [A, 1]$. So there is exactly one root in the interval $[A, 1]$. As once again $f(T^*) = \frac{A}{C}(B - 1)(T^* - \frac{B}{B - 1})(1 - T^*) + T^{*2}(T^* - A)$ is positive for $T^* \in [1, \frac{B}{B - 1}]$, there is exactly one feasible root in this case.

If $B = 1$, we have $f(T^*) = T^{*2}(T^* - A) + \frac{A}{C}(T^* - 1) > 0$ for $T^* > 1$ and $f'(T^*) = T^{*2} + 2T^*(T^* - A) + \frac{A}{C} > 0$ for $T^* \in [A, 1]$. So there is exactly one feasible root in this case too.

4. case $A < 1$ and $B < 1$.

As $f(1) = 1 - A > 0$ and $f(A) = (A - 1)\frac{A}{C}(A(1 - B) + B) < 0$, there is at least one feasible root in this case too. Let $\tilde{T} = T^* - A$ and $\tilde{f}(\tilde{T}) = f(\tilde{T} + A)$. Then the equation $\tilde{f}(\tilde{T}) = 0$ is written as:

$$\begin{aligned} \tilde{T}^3 + \left(3A + \frac{A}{C}(1 - B - C)\right)\tilde{T}^2 + \left(3A^2 + 2\frac{A^2}{C}(1 - B - C) + \frac{A}{C}(2B - 1)\right)\tilde{T} \\ + \frac{A}{C}(A - 1)(A(1 - B) + B) \\ = \tilde{T}^3 + \left(2A + \frac{A}{C}(1 - B)\right)\tilde{T}^2 + \left(A^2 + 2\frac{A^2}{C}(1 - B) + \frac{A}{C}(2B - 1)\right)\tilde{T} \\ + \frac{A}{C}(A - 1)(A(1 - B) + B) = 0. \end{aligned}$$

Using *Descartes' rule of signs* we can see that there is exactly one positive root $\tilde{T}$ of the cubic equation $\tilde{f}(\tilde{T}) = 0$ as the coefficients of $\tilde{T}^3$ and $\tilde{T}^2$ are positive and the constant term is negative. So there is exactly one feasible root $T^* \geq A$ in this case too.

Let us summarize. The feasible equilibria points of the system can be classified as follows:

- Disease-free equilibrium: $(S^*, N^*, M^*, T^*) = (0, 1, 1, 1)$.
- Other equilibria with $S^* = 0$: $S^* = 0, N^* = 0, M^* = 0, T^* = R$, with arbitrary R ; $S^* = 0, N^* = 0, M^* = 1, T^* = 1$; $S^* = 0, N^* = 1, M^* = 0, T^* = R$, with arbitrary R .
- Feasible equilibria with $S^* \neq 0$: None when $A \geq 1$. When $A < 1$, there is only one biologically feasible equilibria with $S^* \neq 0$ (see proof and Figure 6).

2.2 Stability analysis

Stability analysis of the equilibria point (S^*, N^*, M^*, T^*) is done by linearizing the system around this point. The Jacobian matrix of the system is given by:

$$J = \begin{pmatrix} aN - bT & aS & 0 & -bS \\ \alpha N & \alpha S - \delta_N(2N - 1) & 0 & 0 \\ \beta & 0 & -\delta_M(2M - 1) & 0 \\ -\gamma T & 0 & \delta_M(1 - T) & -\gamma S - \delta_M M \end{pmatrix}$$

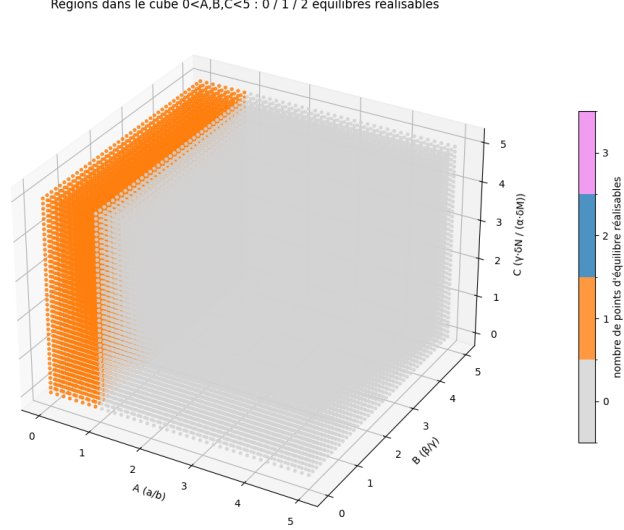


Figure 6: Feasible region in the (A, B, C) space.

2.2.1 Stability of inflammation free equilibria ($S^* = 0$).

We analyze first the stability of the disease-free equilibrium point $(S^*, N^*, M^*, T^*) = (0, 1, 1, 1)$. At this point, the Jacobian matrix simplifies to:

$$J|_{(S^*, N^*, M^*, T^*)=(0,1,1,1)} = \begin{pmatrix} a-b & 0 & 0 & 0 \\ \alpha & -\delta_N & 0 & 0 \\ \beta & 0 & -\delta_M & 0 \\ -\gamma & 0 & 0 & -\delta_M \end{pmatrix}$$

We notice immediately that the eigenvalues of this matrix are $a - b$, $-\delta_N$ and $-\delta_M$. The last two are negative since $\delta_N, \delta_M > 0$. The remaining eigenvalue is $a - b$. Thus the disease-free equilibrium is stable if $a < b$ and unstable if $a > b$. This means that if the inflammatory effect of Neutrophils cells controlled by parameter a is less than the curing effect of the SL-TRM macrophages controlled by parameter b , the system will return to the disease-free state. Conversely, if $a > b$, the system may move away from this equilibrium, potentially leading to a chronic inflammatory state. The stability of other equilibria points can be analyzed similarly by evaluating the Jacobian matrix at those points and determining the eigenvalues.

For $S^* = 0, N^* = 0, M^* = 0, T^* = R$, with arbitrary R , the Jacobian matrix becomes:

$$J|_{(S^*, N^*, M^*, T^*)=(0,0,0,R)} = \begin{pmatrix} -bR & 0 & 0 & 0 \\ 0 & \delta_N & 0 & 0 \\ \beta & 0 & \delta_M & 0 \\ -\gamma R & 0 & \delta_M(1-R) & 0 \end{pmatrix}$$

The eigenvalues of this matrix are $-bR$, δ_N , δ_M , and 0. Since δ_N and δ_M are positive, this equilibrium point is unstable. Even if the immune signal will decrease exponentially fast, the populations of neutrophils and momac will grow out of bound. This unstable equilibrium will not be observed in practice.

For $S^* = 0, N^* = 0, M^* = 1, T^* = 1$, the Jacobian matrix is:

$$J|_{(S^*, N^*, M^*, T^*)=(0,0,1,1)} = \begin{pmatrix} -b & 0 & 0 & 0 \\ 0 & \delta_N & 0 & 0 \\ \beta & 0 & -\delta_M & 0 \\ -\gamma & 0 & 0 & -\delta_M \end{pmatrix}$$

The eigenvalues of this matrix are $-b$, δ_N , $-\delta_M$, and $-\gamma$. Since there is a positive eigenvalue, this equilibrium point is unstable. Neutrophils can grow out of bound. This equilibrium will not be observed in practice neither.

For $S^* = 0, N^* = 1, M^* = 0, T^* = R$, with arbitrary R , the Jacobian matrix is

$$J|_{(S^*, N^*, M^*, T^*)=(0,1,0,R)} = \begin{pmatrix} a - bR & 0 & 0 & 0 \\ \alpha & -\delta_N & 0 & 0 \\ \beta & 0 & \delta_M & 0 \\ -\gamma R & 0 & \delta_M(1 - R) & 0 \end{pmatrix}$$

There is a positive eigenvalue δ_M , hence this equilibrium point is unstable. Momac can grow out of bound. This equilibrium will not be observed in practice neither.

2.2.2 Stability of equilibrium with $S^* \neq 0$.

The stability of the equilibria with $S^* \neq 0$ can be analyzed similarly by evaluating the Jacobian matrix at those points and determining the eigenvalues. The computations are somewhat involved and will be done numerically for given values of the parameters A , B , and C . We proved that biologically feasible equilibrium with $S^* \neq 0$ only exists if $A < 1$ and is unstable under a threshold $A_0 < 1$ very close to 1, independent of B and C (to be proved rigorously). For $A \geq 1$ the only biologically feasible equilibrium is the disease-free equilibrium which is unstable since $a > b$.

2.3 Bifurcation analysis

We perform a bifurcation analysis with respect to the *non dimensional* parameters $A = a/b$. From the stability analysis, we see that we have bifurcations at $A = A_0$ and $A = 1$: for $A < A_0 \approx 0.998$ there are two biologically feasible equilibrium, one inflammatory one with $S^* \neq 0$ which is unstable and the disease-free equilibrium which is stable; between $A = A_0$ and $A = 1$ there are two equilibria, one inflammatory and one disease-free equilibrium, both are stable; for $A > 1$ the disease-free equilibrium is unstable and there is no other biologically feasible equilibrium. The number and stability of equilibria change as the parameter A crosses these critical values, indicating bifurcation points in the system's dynamics. We can plot the bifurcation diagram in the (A, S^*) plane (see Figure 2.3).

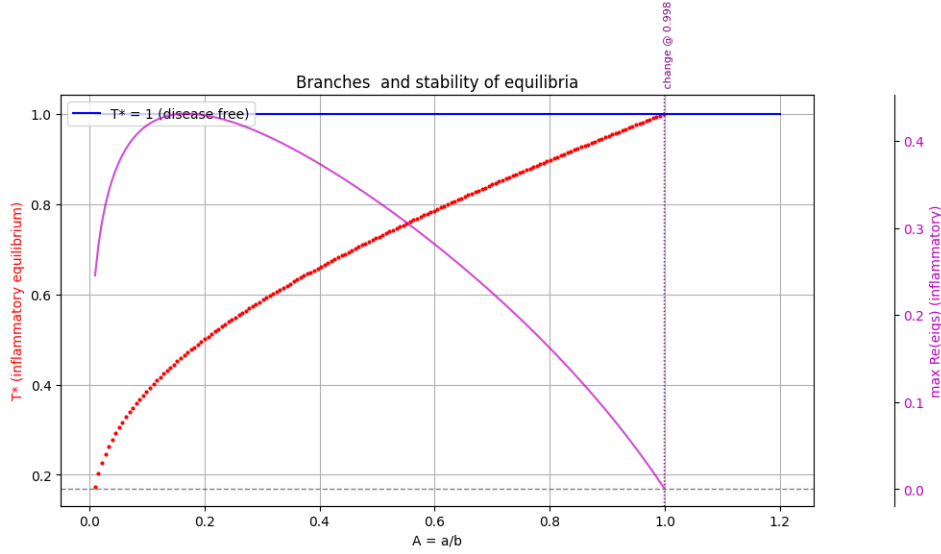


Figure 7: Bifurcation diagram in the (A, T^*) plane.

To Do List

- perform stability analysis of non homeostatic equilibria (chronic inflammation)
- Perform bifurcation analysis and plot results
- investigate possibility of limit cycles
- is there hysteresis phenomenon?
- Plot 4d phase space trajectories (projections)
- Run numerical simulations with realistic rates
- Discuss model assumptions/results with immunology collaborators
- use optimal control theory to model treatments (corticosteroids, anti TNF alpha, etc.)

3 Numerical simulations

Here we reproduce one typical simulations of the model using the following parameters: $\alpha = 1, \beta = 1, \gamma = 1, \delta_N = 1, \delta_M = 1, a = 0.35, b = 1$, duration=20 days and initial conditions $S(0) = 1, N(0) = 1, M(0) = 1, T(0) = 1$. We can plot a phase space trajectory in the (S, N, M, T) 4D space, using color intensity to represent the S variable in fig. 9. We can also plot some 2D projections of the 4D phase space trajectory using the tesseract representation in fig. 10.

We need to have realistic values for the parameters to feed the model.

The Python Jupyter notebook used to perform the simulations is available at <https://github.com/immunology/immunology>:

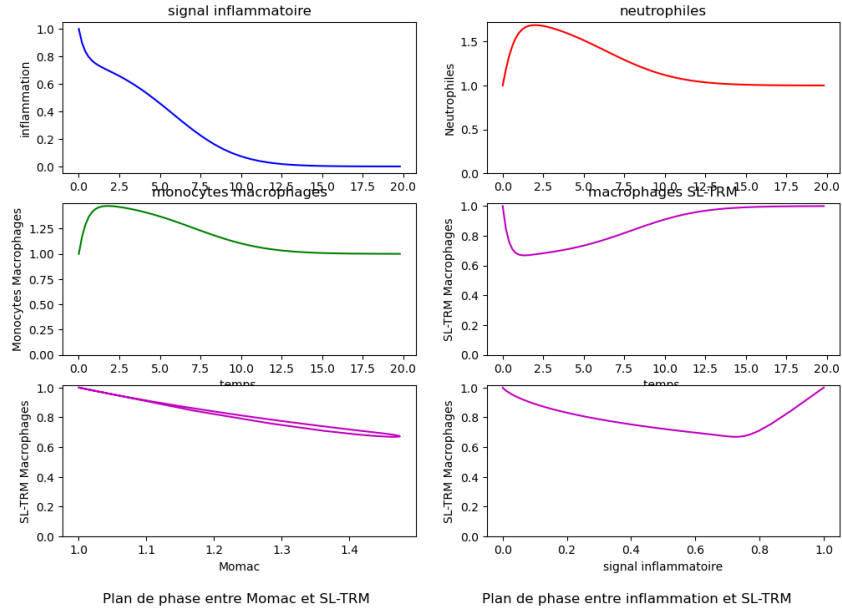


Figure 8:

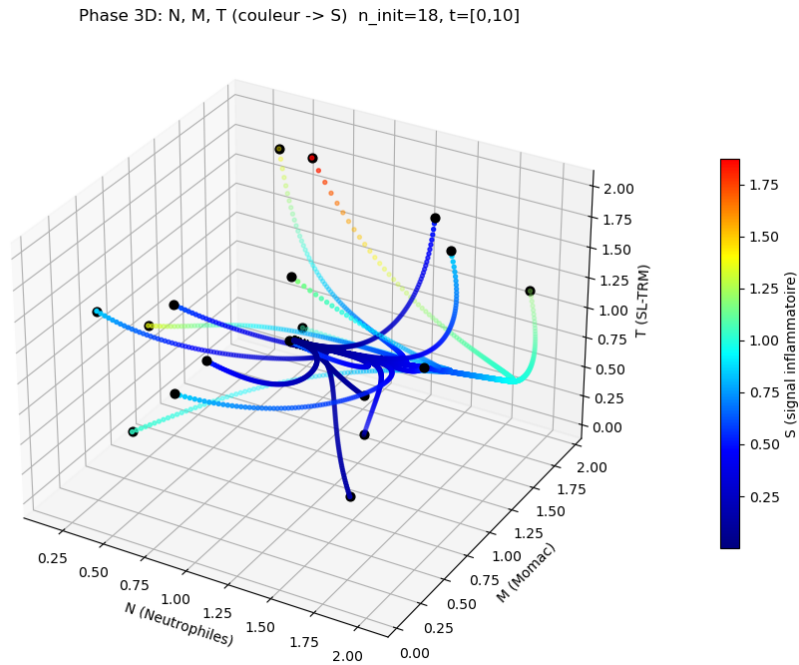


Figure 9: Phase space trajectory in the (S, N, M, T) space, S intensity corresponds to the color scale.

2D projection of 4D points with tesseract (hypercube) reference

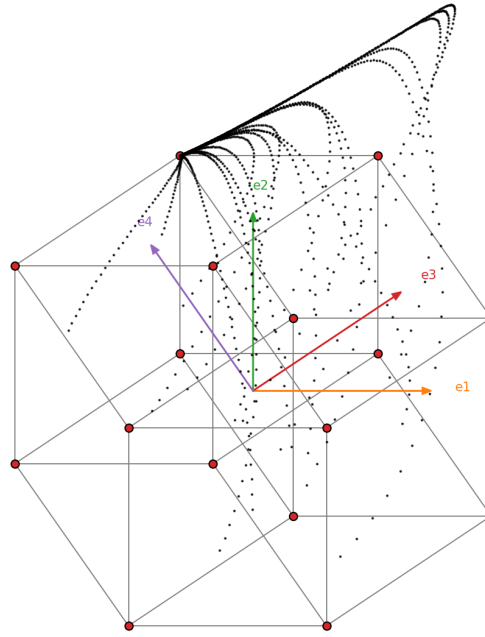


Figure 10: 2D projections of the 4D phase space trajectory using the tesseract representation.

[//github.com/azerad/immunomath](https://github.com/azerad/immunomath).

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References

- [1] Jit, M., et al. (2005). Mathematical modelling of rheumatoid arthritis. *Rheumatology*, 44, 323–331. <https://doi.org/10.1093/rheumatology/keh491>
- [2] Baker, M., et al. (2013). Mathematical modelling of cytokine-mediated inflammation in rheumatoid arthritis *Mathematical Medicine and Biology*, 30, 311–337. <https://doi.org/10.1093/imamb/dqs026>
- [3] Baker, M., et al. (2017). Mathematical modelling of cytokines, MMPs and fibronectin fragments in osteoarthritic cartilage *Journal of Mathematical Biology*, 75, 985–1024 (2017). <https://doi.org/10.1007/s00285-017-1104-y>
- [4] Baker, M., et al. (2018). Mathematical models of inflammation and its resolution. *Immunology Letters*, 196, 1–10. <https://doi.org/10.1016/j.imlet.2018.01.005>
- [5] Zhou, X., et al. (2018). Circuit Design Features of a Stable Two-Cell System. *Cell*, 172, 744–757. <https://doi.org/10.1016/j.cell.2018.01.015>

- [6] Odisharia, K., Odisharia, V., Tsereteli, P., and Janikashvili, N. (2019). On the Mathematical Model of Drug Treatment of Rheumatoid Arthritis In G. Jaiani and D. Natroshvili (eds.). Mathematics, Informatics, and their Applications in Natural Sciences and Engineering. *Springer Proceedings in Mathematics & Statistics*, 2019. https://doi.org/10.1007/978-3-030-10419-1_10
- [7] Handel, A., et al (2020). Simulation modelling for immunologists *Nature Reviews Immunology*, 20, 186–195. <https://doi.org/10.1038/s41577-019-0235-3>
- [8] Macfarlane, F. R., Chaplain, M. A. J., & Eftimie, R. (2022). Mathematical models of immune-mediated diseases: A review. *Mathematics*, 10(3), 456.
- [9] Ugolkov, Y., et al. (2024). Mathematical modeling in autoimmune diseases: from theory to clinical application *Frontiers in Immunology*, 15, 1371620. <https://doi.org/10.3389/fimmu.2024.1371620>